Blank Scenario Template and Guide

TRAUMA/ENVIRONMENTAL: 24-year-old male with hot pavement burns and heat exhaustion that may progress to heat stroke

Scenario Set Up	*Equipment: Gloves, burn moulage, cool water, sterile dry dressings, fan, BP cuff, stethoscope, pulse oximeter, glucometer, and stretcher.* *PROCTOR: You are working at an outdoor barbecue on a hot day. You accidentally press your right forearm against the hot metal grill. You have also been standing in the sun for several hours with little water and feel sweaty, thirsty, weak, and slightly dizzy.* *Sit in a chair, hold your injured arm, and say, "I burned my arm on the grill. I'm also feeling a little overheated."* *Remain alert and oriented throughout. There is no deterioration or heat-stroke branch. Your dizziness improves after rest and cooling, but your burn remains painful. A coworker called 911.*
Dispatch	*"Respond for a 24-year-old male with a burn to his right forearm at an outdoor barbecue. Patient is awake and talking."*
Scene Size Up	*Patient is seated near the grill, holding his right arm.* *The grill remains hot; move treatment to a safe, shaded area.* *One patient, no major bleeding, no fire or smoke exposure.* *No fall or other mechanism of injury.*
Pertinent Primary Assessment Findings	***AVPU:*** *Alert.* ***A&O:*** *×4—person, place, time, and event.* ***Airway:*** *Patent; speaking in full sentences.* ***Breathing:*** *Regular and unlabored; clear, equal lung sounds.*

	Circulation: *Radial pulse slightly rapid and strong. Skin warm and sweaty. No major bleeding; capillary refill under 2 seconds.*
Pertinent Secondary Assessment Findings	***SAMPLE*** ● ***S:*** *Forearm pain, thirst, sweating, mild weakness, and dizziness. No nausea, confusion, chest pain, or difficulty breathing.* ● ***A:*** *No known allergies.* ● ***M:*** *None.* ● ***P:*** *No significant medical history.* ● ***L:*** *Lunch three hours ago; little water since.* ● ***E:*** *Accidentally contacted the hot grill after working outside for several hours.* ***OPQRST*** ● ***O:*** *Immediately after touching the grill.* ● ***P:*** *Worse with touch; improves somewhat with cooling.* ● ***Q:*** *Burning and stinging.* ● ***R:*** *Limited to the right forearm.* ● ***S:*** *6/10.* ● ***T:*** *Approximately 10 minutes before arrival.* ***Focused examination / DCAP-BTLS*** ● ***Right forearm:*** *A palm-sized burn on the inner surface, midway between the elbow and wrist. Red, moist, tender skin with several blisters and mild swelling.* ***Partial-thickness burn, approximately 1% TBSA.*** ● *Burn does not encircle the arm or cross a joint. No stuck clothing.* ● *Right radial pulse, finger movement, and sensation intact.* ● *No deformities, contusions, abrasions, punctures, or lacerations.* ● *Head, neck, chest, abdomen, back, pelvis, and remaining extremities: no injuries or tenderness.*

Vitals	| *Finding* | *Initial* | *After rest and cooling* | | *BP* | *110/72* | *116/76* | | *HR* | *104* | *92* | | *RR* | *20* | *16* | | *SpO_2* | *98% room air* | *98% room air* | | *BGL* | *100 mg/dL* | *Not repeated* | | *Mental status* | *A&O ×4* | *A&O ×4* | | *Burn pain* | *6/10* | *4/10* |
Treatments	*Move away from the grill into shade or a cool environment.* *Cool the small burn with cool running water for up to 20 minutes without delaying transport; then apply a dry sterile dressing. Remove jewelry before swelling develops. Avoid ice directly on the burn and leave blisters intact.* *Rest the patient, loosen excess clothing, and fan. Small amounts of water may be given because he is fully alert, can swallow, and has no nausea.* *Reassess the burn, distal circulation, mental status, and vitals; arrange transport for evaluation.*
Key Points	● *The **burn is the main injury**.* ● *Heat exhaustion is a straightforward secondary problem that improves with cooling.* ● *The patient stays oriented and cooperative throughout.* ● *Prevention: avoid hot surfaces, use appropriate protective equipment, hydrate, and take shaded breaks.*
Bonus Questions	***How is burn size estimated here?** The patient's palm and fingers together represent approximately 1% TBSA.*

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MEDICAL: 20-year-old female with difficulty breathing after a bee sting

Scenario Set Up	*Equipment: Gloves, eye protection, EpiPen trainer labeled as the patient's prescribed 0.3 mg EpiPen, oxygen, NRB mask, BVM, suction, BP cuff, stethoscope, pulse oximeter, glucometer, and stretcher.* *PROCTOR: You are at a park having a picnic with a friend. Approximately 10 minutes ago, a bee stung your left forearm. You have a known bee allergy and carry a prescribed EpiPen, but you have not used it because you were hoping the symptoms would go away.* *Act anxious, scratch your arms, and speak in short phrases. Say, "My throat feels tight, and it's getting hard to breathe." Your friend called 911 and knows the EpiPen is in your backpack.* *If students provide prompt treatment, gradually speak more comfortably. If treatment is delayed, become increasingly breathless and less responsive.*
Dispatch	*"You are dispatched to a park for a 20-year-old female experiencing difficulty breathing. A friend is on scene."*
Scene Size Up	● *Patient is seated on a picnic bench, leaning forward and visibly distressed.* ● *Hives are visible on her arms and neck.* ● *Her friend is standing nearby with her backpack.* ● *Bees are still near an open drink at the table; assess whether relocation is needed.* ● *One patient; no traumatic injuries or major bleeding.* ● *Request ALS support early.*
Pertinent Primary Assessment Findings	*AVPU: Alert.* *A&O: ×4 to person, place, time, and event.* *Airway: Open, but threatened by swelling. Voice is hoarse, with* *mild lip swelling and reported throat tightness.*

	Breathing: Rapid and labored at 28/min, bilateral wheezing, *speaking in short phrases.* *Circulation: 118 Rapid, weak radial pulse. Skin is pale and* *clammy with widespread hives. Capillary refill approximately 3* *seconds.* *Priority: High-priority patient with suspected anaphylaxis.*
Pertinent Secondary Assessment Findings	***SAMPLE*** ● ***S:*** *Difficulty breathing, throat tightness, itching, hives, and dizziness.* ● ***A:*** *Bee venom; previous severe reaction.* ● ***M:*** *Prescribed EpiPen 0.3 mg as needed for anaphylaxis. No dose taken today.* ● ***P:*** *No asthma or cardiac history.* ● ***L:*** *Sandwich and water approximately 30 minutes ago.* ● ***E:*** *Stung while eating outdoors; symptoms worsened over several minutes.* ***OPQRST*** ● ***O:*** *Began shortly after the sting.* ● ***P:*** *Progressively worsening without treatment.* ● ***Q:*** *"My throat feels like it's closing."* ● ***R:*** *Tightness in throat and chest.* ● ***S:*** *Breathing difficulty rated 8/10.* ● ***T:*** *Sting occurred approximately 10 minutes ago.* ***Focused examination:*** *Small sting site on left forearm with no retained stinger visible. Hives extend across the torso. No evidence of a fall or other injury.*
Vitals	*INITIAL: BP: 92/58, HR:120, RR:28,labored, BGL: 104, SPO2: 92,* *After prompt epinephrine and supportive care : BP: 106/68, HR: 118, RR: 22 improving, SpO2: 97 with oxygen,*

Treatments	*Promptly administer the patient's 0.3 mg EpiPen IM into the lateral thigh, after medication checks; use a trainer during simulation.* *Provide oxygen, airway support, and BVM assistance if needed.* *Transport promptly and reassess frequently. ALCO allows one repeat IM epinephrine dose after 15 minutes if indicated.*
Key Points	*EpiPen administration is a required action.* *Do not allow a lengthy history to delay treatment.* *The friend provides medication information if the patient cannot finish sentences.*
Bonus Questions	*What are you allergic to? (go around circle asking if time permits)*